Patient Record — ID 1698

Age	32	**Sex**	F
Race/Ethnicity	Non-Hispanic White	**Insurance**	Blue Cross Blue Shield PPO
Total Encounters	8	**Generation Seed**	3940

Primary Diagnoses

• Postpartum blues (Baby blues)
• Coccydynia (musculoskeletal pelvic pain)
• Postpartum depression
• Stress urinary incontinence
• Femoral nerve neuropathy
• Rectovaginal fistula
• External anal sphincter injury

Comorbidities

• Exclusive breastfeeding
• Pregnancy (20 weeks gestation)
• Primiparity

Patient Profile

Sex	F
Age at First Encounter	32
Race/Ethnicity	Non-Hispanic White
Insurance	Blue Cross Blue Shield PPO
Occupation	Elementary school teacher
Smoking Status	Never smoker
Alcohol Use	None (postpartum/breastfeeding)

Allergies:

• NKDA

Chronic Conditions:

None

Home Medications:

None

Surgical History:

• Wisdom teeth extraction (age 22)

Family History:

• Mother: Postpartum depression following second pregnancy
• Father: Hypertension

• Sibling: None

Encounters (8)

Encounter 1 — 2020-01-15 — Obstetrics & Gynecology

Encounter ID	6943	**Type**	inpatient
Attending	Dr. Sarah Chen	**Order**	0
Department	Obstetrics & Gynecology	**Gen. Method**	template

Chief Complaint:

Perineal pain and sensation of incomplete bowel evacuation immediately following vaginal delivery with episiotomy

Full Note:

```
ENCOUNTER NOTE
Date: 2020-01-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Perineal pain and sensation of incomplete bowel evacuation immediately following vaginal delivery
with episiotomy

HISTORY OF PRESENT ILLNESS:
Undergoes spontaneous vaginal delivery at 39 weeks gestation. Due to concern about shoulder
dystocia during the second stage of labor, a midline episiotomy is performed to widen the vaginal
outlet. The delivery is otherwise uncomplicated, and a healthy newborn is delivered with good
Apgar scores.

PAST SURGICAL HISTORY:
- Wisdom teeth extraction (age 22)

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Postpartum depression following second pregnancy
- Father: Hypertension
- Sibling: None

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (postpartum/breastfeeding). Occupation: Elementary school
teacher.

PHYSICAL EXAMINATION:
Postpartum examination reveals a clean surgical extension of the perineal incision and no vaginal
lacerations. On examination, the perineal incision is intact, but there is decreased anal
sphincter tone and minimal voluntary contraction with digital rectal exam. No visible mucosal
lacerations are seen. There is no evidence of rectovaginal fistula. The remainder of the pelvic
exam is normal, and the uterus is firm and well-contracted.
```

Encounter 2 — 2020-01-15 — Obstetrics & Gynecology

Encounter ID	6936	**Type**	outpatient
Attending	Dr. Priya Patel	**Order**	1
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine postpartum visit with tearfulness, irritability, and difficulty sleeping 10 days after delivery

Full Note:

```
ENCOUNTER NOTE
Date: 2020-01-15
Provider: Dr. Priya Patel, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine postpartum visit with tearfulness, irritability, and difficulty sleeping 10 days after
delivery

HISTORY OF PRESENT ILLNESS:
The patient returns for routine postpartum evaluation approximately 10 days following vaginal
delivery with episiotomy complicated by external anal sphincter injury. She gave birth to a
healthy baby girl 10 days ago and reports feeling overwhelmed and tearful since delivery, with
episodes of unexplained crying and irritability. She also mentions difficulty sleeping and
concentrating, which she attributes to the new responsibilities of motherhood. She denies any
thoughts of harming herself or her baby.

PAST MEDICAL HISTORY:
Active Problem List:
- External anal sphincter injury (diagnosed 2020-01-15)

Her pregnancy was uncomplicated.

PAST SURGICAL HISTORY:
- Wisdom teeth extraction (age 22)

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Postpartum depression following second pregnancy
- Father: Hypertension
- Sibling: None

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (postpartum/breastfeeding). Occupation: Elementary school
teacher.

VITAL SIGNS:
Her vital signs are stable

PHYSICAL EXAMINATION:
There are no abnormal findings

LABORATORY DATA:
Her complete blood count and thyroid function tests are within normal limits.
```

Encounter 3 — 2020-02-15 — Obstetrics & Gynecology

Encounter ID	6937	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	2
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Persistent localized pain at the base of the spine for two weeks, exacerbated by sitting, following vaginal delivery three weeks ago

Full Note:

```
ENCOUNTER NOTE
Date: 2020-02-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Persistent localized pain at the base of the spine for two weeks, exacerbated by sitting,
following vaginal delivery three weeks ago

HISTORY OF PRESENT ILLNESS:
The patient returns for evaluation of persistent pain three weeks following her vaginal delivery,
with recent history of external anal sphincter injury and postpartum blues. She reports that the
pain is localized at the base of her spine and is exacerbated by sitting down or transitioning
from sitting to standing. She denies any recent trauma or injury to the area. She gave birth to
her first child via vaginal delivery three weeks ago. The delivery was complicated by a prolonged
second stage of labor.

PAST MEDICAL HISTORY:
Active Problem List:
- External anal sphincter injury (diagnosed 2020-01-15)
- Postpartum blues (Baby blues) (diagnosed 2020-01-15)

PAST SURGICAL HISTORY:
- Wisdom teeth extraction (age 22)

MEDICATIONS:
She is not taking any medications

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Postpartum depression following second pregnancy
- Father: Hypertension
- Sibling: None

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (postpartum/breastfeeding). Occupation: Elementary school
teacher.

PHYSICAL EXAMINATION:
On physical examination, there is tenderness to palpation over the coccyx. No overlying skin
changes or masses are noted. Rectal examination is deferred due to patient discomfort.
```

Encounter 4 — 2020-02-15 — Obstetrics & Gynecology

Encounter ID	6939	**Type**	outpatient
Attending	Dr. Aisha Johnson	**Order**	3
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Involuntary urinary leakage with coughing or laughing for four weeks since vaginal delivery

Full Note:

```
ENCOUNTER NOTE
Date: 2020-02-15
Provider: Dr. Aisha Johnson, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Involuntary urinary leakage with coughing or laughing for four weeks since vaginal delivery

HISTORY OF PRESENT ILLNESS:
The patient returns to the obstetrics clinic for further evaluation of postpartum symptoms
following vaginal delivery complicated by external anal sphincter injury and recent coccydynia.
She gave birth to a healthy baby girl vaginally 4 weeks ago. She reports that since the delivery,
she has experienced involuntary leakage of urine when she coughs or laughs. She denies any burning
with urination or increased frequency.

PAST MEDICAL HISTORY:
Active Problem List:
- External anal sphincter injury (diagnosed 2020-01-15)
- Postpartum blues (Baby blues) (diagnosed 2020-01-15)
- Coccydynia (musculoskeletal pelvic pain) (diagnosed 2020-02-15)

Her prenatal course was uncomplicated.

PAST SURGICAL HISTORY:
- Wisdom teeth extraction (age 22)

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Postpartum depression following second pregnancy
- Father: Hypertension
- Sibling: None

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (postpartum/breastfeeding). Occupation: Elementary school
teacher.

PHYSICAL EXAMINATION:
On physical examination, there is no evidence of urinary tract infection or pelvic organ prolapse.
A cough stress test is positive for urine leakage.
```

Encounter 5 — 2020-03-15 — Obstetrics & Gynecology

Encounter ID	6938	**Type**	outpatient
Attending	Dr. Aisha Johnson	**Order**	4
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine 6-week postpartum visit with persistent feelings of hopelessness, guilt, and anhedonia

Full Note:

```
ENCOUNTER NOTE
Date: 2020-03-15
Provider: Dr. Aisha Johnson, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine 6-week postpartum visit with persistent feelings of hopelessness, guilt, and anhedonia

HISTORY OF PRESENT ILLNESS:
The patient returns for her routine six-week postpartum visit following vaginal delivery
complicated by external anal sphincter injury, having previously presented with postpartum blues
and ongoing pelvic floor dysfunction including coccydynia and stress urinary incontinence. She
reports feeling "down" and "hopeless" most days since the delivery, with difficulty sleeping even
when her baby is asleep. She admits to having little interest in activities she used to enjoy and
feels overwhelmed and guilty about not being a good mother. She has difficulty concentrating and
making decisions, and her appetite has decreased significantly. She tearfully confesses that she
has had thoughts of harming herself but has not made any plans. She denies any thoughts of harming
her baby.

PAST MEDICAL HISTORY:
Active Problem List:
- External anal sphincter injury (diagnosed 2020-01-15)
- Postpartum blues (Baby blues) (diagnosed 2020-01-15)
- Coccydynia (musculoskeletal pelvic pain) (diagnosed 2020-02-15)
- Stress urinary incontinence (diagnosed 2020-02-15)

She had an uncomplicated pregnancy and vaginal delivery.

PAST SURGICAL HISTORY:
- Wisdom teeth extraction (age 22)

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Postpartum depression following second pregnancy
- Father: Hypertension
- Sibling: None

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (postpartum/breastfeeding). Occupation: Elementary school
teacher.

VITAL SIGNS:
Blood pressure: 120/78 mmHg
- Heart rate: 78 bpm
- Respiratory rate: 16 breaths/min
- Temperature: 98.6°F (37°C)
```

PHYSICAL EXAMINATION:
Her physical examination is normal.

LABORATORY DATA:
Hemoglobin: 12.8 g/dL
- Thyroid-stimulating hormone (TSH): 2.5 mIU/L
- Fasting glucose: 92 mg/dL

Encounter 6 — 2020-03-15 — Obstetrics & Gynecology

Encounter ID	6942	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	5
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Six-week history of fecal incontinence and malodorous tan-brown vaginal discharge with gas bubbles

Full Note:

```
ENCOUNTER NOTE
Date: 2020-03-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Six-week history of fecal incontinence and malodorous tan-brown vaginal discharge with gas bubbles

HISTORY OF PRESENT ILLNESS:
She returns postpartum with sphincter injury. She reports that the discharge is tan-brown and
occasionally contains gas bubbles.

PAST MEDICAL HISTORY:
Active Problem List:
- External anal sphincter injury (diagnosed 2020-01-15)
- Postpartum blues (Baby blues) (diagnosed 2020-01-15)
- Coccydynia (musculoskeletal pelvic pain) (diagnosed 2020-02-15)
- Stress urinary incontinence (diagnosed 2020-02-15)
- Postpartum depression (diagnosed 2020-03-15)

She gave birth to her first child vaginally 8 weeks ago and sustained a fourth-degree perineal
laceration, which was repaired immediately postpartum.

PAST SURGICAL HISTORY:
- Wisdom teeth extraction (age 22)

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Postpartum depression following second pregnancy
- Father: Hypertension
- Sibling: None

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (postpartum/breastfeeding). Occupation: Elementary school
teacher.

VITAL SIGNS:
Vital signs are within normal limits.

PHYSICAL EXAMINATION:
On examination, the patient appears well-nourished and in no acute distress.
Pelvic examination reveals a small amount of tan-brown discharge in the vaginal vault and a dark
red, velvety area of tissue on the posterior vaginal wall. No masses are palpable, and the
remainder of the pelvic examination is unremarkable.
```

Encounter 7 — 2025-01-15 — Obstetrics & Gynecology

Encounter ID	6940	**Type**	inpatient
Attending	Dr. Michael Rodriguez	**Order**	6
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Acute difficulty walking and numbness over the anteromedial thigh 24 hours after vaginal delivery

Full Note:

```
ENCOUNTER NOTE
Date: 2025-01-15
Provider: Dr. Michael Rodriguez, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Acute difficulty walking and numbness over the anteromedial thigh 24 hours after vaginal delivery

HISTORY OF PRESENT ILLNESS:
The multiparous patient with history of prior sphincter injury and fistula is 24 hours postpartum
following vaginal delivery of a healthy baby boy complicated by prolonged second stage of labor
requiring hyperflexion of her hips.

PAST MEDICAL HISTORY:
Active Problem List:
- External anal sphincter injury (diagnosed 2020-01-15)
- Postpartum blues (Baby blues) (diagnosed 2020-01-15)
- Coccydynia (musculoskeletal pelvic pain) (diagnosed 2020-02-15)
- Stress urinary incontinence (diagnosed 2020-02-15)
- Postpartum depression (diagnosed 2020-03-15)
- Rectovaginal fistula (diagnosed 2020-03-15)

PAST SURGICAL HISTORY:
- Wisdom teeth extraction (age 22)

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Postpartum depression following second pregnancy
- Father: Hypertension
- Sibling: None

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (postpartum/breastfeeding). Occupation: Elementary school
teacher.

PHYSICAL EXAMINATION:
On physical examination, she has a decreased sensation over the anteromedial aspect of her right
thigh and weakness during hip flexion and knee extension on the same side. Deep tendon reflexes
are normal, and there is no back pain or tenderness.
```

Encounter 8 — 2025-02-15 — Obstetrics & Gynecology

Encounter ID	6941	**Type**	outpatient
Attending	Dr. Emily Watson	**Order**	7
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine 6-week postpartum visit with low mood, sleep disturbance, and irritability

Full Note:

```
ENCOUNTER NOTE
Date: 2025-02-15
Provider: Dr. Emily Watson, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Routine 6-week postpartum visit with low mood, sleep disturbance, and irritability

HISTORY OF PRESENT ILLNESS:
The patient is a 37-year-old female returning for her routine 6-week postpartum visit following a
delivery complicated by femoral nerve neuropathy, with a history of postpartum depression after
her 2020 delivery. She reports feeling "down" and has trouble sleeping, even when her baby is
asleep. She expresses concern about her lack of interest in activities she used to enjoy and
admits to feeling overwhelmed and irritable. She denies any thoughts of harming herself or her
baby.

PAST MEDICAL HISTORY:
Active Problem List:
- External anal sphincter injury (diagnosed 2020-01-15)
- Postpartum blues (Baby blues) (diagnosed 2020-01-15)
- Coccydynia (musculoskeletal pelvic pain) (diagnosed 2020-02-15)
- Stress urinary incontinence (diagnosed 2020-02-15)
- Postpartum depression (diagnosed 2020-03-15)
- Rectovaginal fistula (diagnosed 2020-03-15)
- Femoral nerve neuropathy (diagnosed 2025-01-15)

Her pregnancy was uncomplicated, and she has no prior history of psychiatric illness.

PAST SURGICAL HISTORY:
- Wisdom teeth extraction (age 22)

ALLERGIES:
NKDA

FAMILY HISTORY:
- Mother: Postpartum depression following second pregnancy
- Father: Hypertension
- Sibling: None

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (postpartum/breastfeeding). Occupation: Elementary school
teacher.

VITAL SIGNS:
Her vital signs are within normal limits.

PHYSICAL EXAMINATION:
On examination, she appears fatigued but is otherwise well-appearing.
```

PLAN:
The physician decides to screen for postpartum depression.